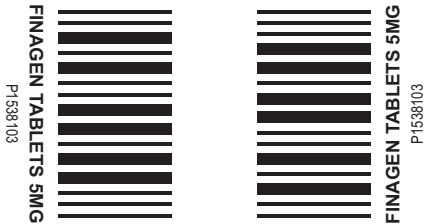


Pharmacode position may change as per the supplier's m/c requirement



FINAGEN TABLETS 5MG

(Finasteride Tablets 5 mg)

Qualitative and Quantitative Composition

Each film coated tablets contains:
Finasteride Ph.Eur 5 mg

Pharmaceutical Form

Blue coloured, circular, biconvex, beveled edged film-coated tablets debossed with 'E' on one side and '61' on the other side.

Pharmacodynamic Properties

Finasteride is a competitive inhibitor of human 5 α reductase, an intracellular enzyme which metabolises testosterone into the more potent androgen, dihydro-testosterone (DHT). In benign prostatic hyperplasia (BPH), enlargement of the prostate gland is dependent upon the conversion of testosterone to DHT within the prostate. Finasteride is highly effective in reducing circulating and intraprostatic DHT. Finasteride has no affinity for the androgen receptor.

Pharmacokinetic Properties:

After an oral dose of 14C - Finasteride in man, 39 % of the dose was excreted in the urine in the form of metabolites (virtually no unchanged drug was excreted in the urine), and 57% of total dose was excreted in the faeces. Two metabolites have been identified which possess only a small fraction of the Type II 5 α -reductase activity of finasteride.

The oral bioavailability of Finasteride is approximately 80 %, relative to an intravenous reference dose, and is unaffected by food. Maximum plasma concentrations are reached approximately two hours after dosing and the absorption is complete within 6-8 hours.

Protein binding is approximately 93 %. Plasma clearance and the volume of distribution are approximately 165 ml/min and 76 L, respectively.

In the elderly, the elimination rate of Finasteride is somewhat decreased. Half-life is prolonged from a mean half-life of approximately six hours in men aged 18 - 60 years to eight hours in men aged more than 70 years.

This is of no clinical significance and does not want a reduction in dosage.

In patients with chronic renal impairment, whose creatinine clearance ranged from 9 - 55 ml/min, the disposition of a single dose of "C - Finasteride was not different from that in healthy volunteers. Protein binding also did not differ in patients with renal impairment. Finasteride is extensively metabolized in the liver, primarily via the cytochrome P450 3A4 enzyme subfamily. Two metabolites, the t-butyl side chain monohydroxylated and monocarboxylic acid metabolites, have been identified that possess no more than 20 % of the 5 α -reductase inhibitory activity of Finasteride.

A portion of the metabolites which normally is excreted renally was excreted in the faeces. It therefore appears that faecal excretion increases commensurate to the decrease in urinary excretion of metabolites. Dosage adjustment in non-dialyzed patients with renal impairment is not necessary.

Indication:

- Finasteride is indicated for the treatment and control of benign prostatic hyperplasia (BPH) and for the prevention of urologic events to:
 - reduce the risk of acute urinary retention.
 - reduce the risk of surgery including transurethral resection of the prostate (TURP) and prostatectomy.

Finasteride causes regression of the enlarged prostate, improves urinary flow and improve the symptoms associated with BPH.

Recommended Dose:

The recommended adult dose is one 5 mg tablet daily with or without food.

Dosage in renal insufficiency

No adjustment in dosage is required in patients with varying degrees of renal insufficiency (creatinine clearances as low as 9 ml/min) as pharmacokinetic studies did not indicate any change in disposition of finasteride

Dosage in the Elderly

No adjustment in dosage is required although pharmacokinetic studies indicated the elimination of finasteride is somewhat decreased in patients more than 70 years of age

Mode of Administration

Oral

Contraindication

Finasteride is not indicated for use in women or children. Finasteride is contraindicated in the following:

- Hypersensitivity to any component of this product;
- Pregnancy - use in women when they are or may potentially be pregnant.

Warning and Precautions:

General

Prior to initiating therapy with Finasteride, appropriate evaluation should be performed to identify other conditions such as infection, prostate cancer, stricture disease, hypotonic bladder or other neurogenic disorders that might mimic BPH. Precaution should be taken before the administration of Finasteride in those patients with liver function abnormalities, as Finasteride is metabolized extensively in the liver.

Patients with large residual urine volume and/or severely diminished urinary flow should be carefully monitored for obstructive uropathy.

Effects on prostate-specific antigen (PSA) and prostate cancer detection

Digital rectal examination, as well as other evaluations for prostate cancer, should be carried out on patients with BPH prior to initiating therapy with Finasteride and periodically thereafter. Generally, when PSA assays are performed a baseline PSA>10 ng/ml (Hybritech) prompts further evaluation and consideration of biopsy; for PSA levels between 4 and 10 ng/ml, further evaluation is advisable. There is considerable overlap in PSA levels among men with and without prostate cancer. Therefore, in men with BPH, PSA values within the normal reference range do not rule out prostate cancer regardless of treatment with Finasteride. A baseline PSA <4 ng/ml does not exclude prostate cancer. Finasteride causes a decrease in serum PSA concentrations by approximately 50% in patients with BPH even in the presence of prostate cancer. This decrease in serum PSA levels in patients with BPH treated with Finasteride should be considered when evaluating PSA data and does not rule out concomitant prostate cancer. This decrease is predictable over the entire range of PSA values, although it may vary in individual patients. In patients treated with Finasteride for six months or more, PSA values should be doubled for comparison with normal ranges in untreated men. This adjustment preserves the sensitivity and specificity of the PSA assay and maintains its ability to detect prostate cancer.

Any sustained increase in PSA levels of patients treated with Finasteride should be carefully evaluated, including consideration of non-compliance to therapy with Finasteride.

Percent free PSA (free to total PSA ratio) is not significantly decreased by Finasteride and remains constant even under the influence of Finasteride. When percent free PSA is used as an aid in the detection of prostate cancer, no adjustment is necessary.

Increased Risk of High-Grade Prostate Cancer

Men aged 55 and over with a normal digital rectal examination and PSA $\leq$ 3.0 ng/mL at baseline taking finasteride 5mg/day in the 7- year Prostate Cancer Prevention Trial (PCPT) had an increased risk of Gleason score 8-10 prostate cancer (finasteride 1.8% vs placebo 1.1%). Similar results were observed in a 4-year placebo-controlled clinical trial with another 5-alpha reductase inhibitor(dutasteride) (1% dutasteride vs 0.5% placebo).

5- alpha reductase inhibitors may increase the risk of development of high-grade prostate cancer. Whether the effect of 5-alpha reductase inhibitors to reduce prostate volume, or study-related factors, impacted the results of these studies has not been established.

Male Breast Cancer

Breast cancer has been reported in men taking finasteride 5 mg tablets during the post-marketing period. Physicians should instruct their patients to promptly report any changes in their breast tissue such as lumps, pain, gynaecosmatia or nipple discharge.

Interaction with other Medicaments

No clinically important drug interactions have been identified. Finasteride does not appear to significantly affect the cytochrome P450 - linked drug metabolising enzyme system. Compounds which have been tested in man include propranolol, digoxin, glibenclamide, warfarin, theophylline, and antipyrine and no clinically meaningful interactions were found.

Other concomitant therapy: Although specific interaction studies were not performed in clinical studies, Finasteride was used concomitantly with ACE inhibitors, alpha-blockers, beta-blockers, calcium channel blockers, cardiac nitrates, diuretics, H2 antagonists, HMG - CoA reductase inhibitors.

Non-steroidal anti-inflammatory drugs (NSAIDs) including aspirin and paracetamol, quinolones and benzodiazepines without evidence of clinically significant adverse interactions.

Pregnancy and Lactation

Pregnancy: Finasteride is contraindicated in women who are or may potentially be pregnant. Because of the ability of Type II 5 α -reductase inhibitors to inhibit conversion of testosterone to dihydrotestosterone, these drugs, including finasteride, may cause abnormalities of the external genitalia of a male fetus when administered to a pregnant woman.

Lactation: Finasteride is not indicated during lactation.

Side Effects

Finasteride is well tolerated; adverse reactions usually have been mild and transient.

Following are the adverse effects observed with Finasteride tablets:

Abnormal ejaculation, breast tenderness, breast enlargement, prostate cancer(high grade),reduced libido, decreased volume of ejaculate, neoplasm of male breast, and hypersensitivity reactions(pruritis, rash, urticaria, swelling of face / lips).

Adverse Events: Post Marketing Experience

Male breast cancer.

A/s. 210 x 300 mm

Color : ■ Black

 AUROBINDO Packaging Development		Product Name	Component	Item Code	Date & Time
		Finagen 5 mg	Leaflet	P1538103	21.10.2024 & 04:10 pm
		Customer / Country	Version No.	Reason Of Issue	Reviewed / Approved by
Malaysia_Unit 3	00	Revision			
Team Leader	Kiran Kumar	Dimensions	No. of Colours : 01		
Initiator	Shirisha	210 x 300 mm	 38103		
Artist: Advnt (Aman)		Pharmacode			
		38103			
Additional Information : Supersede item Code: P1525319					Sign / Date

Symptoms and Treatment of Overdose

No specific treatment of over dosage with Finasteride is recommended

Pharmaceutical particulars

List of excipients:

Lactose Monohydrate, Cellulose Microcrystalline, Sodium Starch Glycolate, Starch Pregelatinised, Docusate Sodium, Magnesium Stearate and Opadry Blue.

Incompatibilities

Not known.

Storage Condition

Do not store above 30°C.

Shelf Life

36 months

Nature and contents of container

2 blisters of 10 tablets each are packed in a printed carton along with a package insert.

3 blisters of 10 tablets each are packed in a printed carton along with a package insert.

10 blisters of 10 tablets each are packed in a printed carton along with a package insert.

Date of Revision: October 2024

Manufactured by:



AUROBINDO

Aurobindo Pharma Limited,
Unit-III, Survey No.: 313 & 314,
Bachupally, Bachupally Mandal,
Medchal - Malkajgiri District,
Telangana State, India.

Product Registration Holder and Imported by:
Unimed Sdn Bhd No.53, Jalan SD 5/2B,
Bandar Sri Damansara,
52200 Kuala Lumpur, Malaysia.